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Name: هبة علي علي احمد National ID: 27605010103283	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
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المریضة تعاني من ورم غير حمید بالثدي و تحتاج الي Supportive and palliative Treatment
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية



هبة
على على احمد بركات
٢٨ ش محمد الصاحب هاشم اغا
الشرابية - القاهرة

١٩٧٦/٠٥/١١ ٢٧٦.٥.١.١.٣٢٨٣
JD3770610

٢٠٢١/١١ ٢٧٦.٥.١.١.٣٢٨٣



حاصله على دبلوم تجاره
أنثى
مستلمة
متروجة
الشحات سعيد السيد ابراهيم
البطاقة سنارية حتى ٢٠٢٨/١١/٢٧



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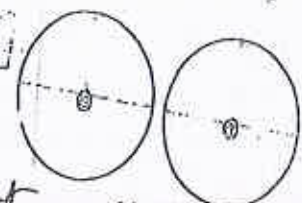


الملكة رانيا العبدالله
مملكة البحرين

Name	1555500/110	MRN	29 11 22
Age	46y.	ID	201A
Menopausal status	premenop	Phone	C.C.
NRS SCORE		marital status	-15555510...
BMI		Family history	CV 2.0. 1.1. 1.5.
Co morbidity		REASON OF MDT	

Clinical Data

Bx:	IDC II				R
ER:	PR:	HER2:	Ki67:		
+	+	-	25%		
Sonomammography: preop RR Breast					
Mets Workup: may RR HER + Recomb					
Multicentric IDC II or 27/7/2022					
T3 (5.5x4) N3 13/16 Mo CR					
T1c1					
Recommendations: for MDT for adjuvant					
BS: pending					
T/CO					



OUTCOME & PLAN

for 70/44/10/10 and have seen
not for me to 1 month till op
of have seen for re-assess

ONCOLOGY	SURGERY	SIGNATURE	RADIOLOGY	PATHOLOGY	Nutrition	psychiatric
29/11/22						



Presidential Initiative for Women's Health - Pathology Lab
المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي

Patient Name : الأستاذة / معدية شوقي فؤيد غنم
Patient No. : 28103182502145
Path No. : 803/2022
Sex : Female
Age : 41 Y
Referring Doc :

Request No. : 13002535
Request Date : 22/03/2022
Verification Date : 05/04/2022
Financial Category : نفقة الدولة
Department : Out Patient
Registered by : marwa mohamed youss

Pathology Report

Clinical Data:

Left breast cancer, Post NACT.

Nature of the specimen:

Left MRM & apical axillary lymph nodes

Gross Examination:

Two specimens were received submitted as:
- Left MRM measuring 22X20X4 cm, with skin covering; 19X10 cm. Cut section revealed an

defined firm greyish white tumor bed site measuring 7x6x5 cm, located 0.5 away from
deep margin.
- Axillary dissection revealed 7 nodules up to 2.5x2x1 cm.

Apical lymph node: Fibrofatty tissue piece measured 6x4x2 cm. entangling 10 nodules
measuring in size from 0.5 cm to 1 cm.

Microscopic examination:

No remnants of tumor tissue

Complete pathological response

Invasion: No

In situ carcinoma: No

Lymph node involvement: Free (0/17)

Section Margins: Free & Free nipple and areola.

Pathologic response to therapy: NA

Additional pathologic findings: Hyalinosis, Fibrosis, necrosis, calcification and sclerotic
ductal papilloma in the surrounding breast tissue

Diagnosis:

Left breast cancer, Left MRM (post neoadjuvant);

Verified by :

Dr. Marwa mahmoud

Date : 05/04/2022 13:52:51

Page 1 of 2

Printing By : marwa mohamed youss

شركة ريف (م) - شارع النيل - حي النور - القاهرة (11511) - مصر
Phone: 011-45555555 - 011-45555555 - 011-45555555
Fax: 011-45555555 - 011-45555555 - 011-45555555
Email: info@rif.com.eg - info@rif.com.eg - info@rif.com.eg



Presidential Initiative for Women's Health - Pathology Lab
المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي

Patient Name : الأستاذة / هبة علي علي احمد بركات
Patient No. : 27605010103283
Path No. : SR-859/22
Sex : Female
Age : 46 Y
Referring Doc :

Request No. : 13006755
Request Date : 07/11/2022
Verification Date : 17/11/2022
Financial Category : أورام دار السلام - مبادره
Department : Out Patient
Registered by : marwa mohamed youss

Pathology Report

- HER-2: Negative (score 0)
- Ki67: 30%

Signature :
Prof. Dr. Manal Elmahdy
Dr. Walaa Ghonaim

Verified by :
walaa ghoniem

Date : 22/11/2022 09:10:02

Page 2 of 2

Printing By : marwa mohamed youss

معمل الباثولوجي - مبادرة صحة المرأة - دار السلام - القاهرة
Room No. (4) - Block 3 - Building No. 237 - Nile Corniche - Near El Giza, Cairo, Egypt

Patient Name: Heba Ali Ali Ahmed Barakat	Date : 20/01/2025
Exam : ultrasound ABDOMEN& PELVIS	Age : 48 Year(s)
Medical ID: 2518/2022	Pat. ID: 97050

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

- enlarged in size (17cm at Rt MCL) bright parenchymal echopattern with smooth surface.
- No hepatic focal lesions or intra hepatic billiary radicle dilatation.
- Portal vein is patent and not dilated.

Gall Bladder: -

- Average distention and capacity with no stones or mud
- The wall is not thickened.
- CBD is not dilated.

Spleen: -

- Average size, homogenous.

Kidneys: -

- Both are average in size, shape and echogenicity
- No stones or dilated pelvicalyceal system
- No gross lesion

Para-aortic and pancreatic area: show no gross abnormalities.

No ascites

Urinary Bladder: -

- Average capacity with no stones or diverticular out pouching

Uterus and adnexal: no abnormalities could be detected

Opinion:

Bright hepatomegaly likely fatty
Colonic gaseous distention

Signature Dr. Nermien Ramadan
20/01/2025 3:52 PM



Patient Name: Heba Ali Ali Ahmed	Date : 31/10/2022
Exam : C.T. ABDOMEN & PELVIS WITH CONTRAST	Age : 46 Year(s)
Medical ID:	

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS REVEALED: -**
**As compared to previous study dated 2/7/2022, the current
study revealed:**

1. Chest:

- Status post operative showing diffuse right breast skin thickening and interstitial edema.
- No pathologically enlarged right axillary lymph nodes
- Still noted left upper lung lobe posterior segment nodule measuring 4 mm (likely insignificant).
- No evidence of other pulmonary nodules or masses.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. Abdomen and pelvis:

- The liver is average in size with homogenous CT texture. No definite focal lesions or contour irregularities.
- No appreciable changes as regards the large right hepatorenal cystic lesion with coarse mural calcifications, it measures 11 x 10 cm seen abutting the adjacent hepatic surface and right renal upper pole.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.

HEBA, Patient ID: 122012598

about:blank

DOB: 01-May-1976 Date: 23-Jan-2023

Patient Lastname: Ali
Patient Firstname: Heba
IIS Patient ID: 122012598
IS Patient ID: 303327
Date of Birth: 01/05/1976

Procedure: 70030, Bone Scan
Medical Question:
Clinical Comment:

REPORT TEXT

mTc MDP Bone Scan Report (23/01/2023)

Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I.V Injection of 20mCi of ^{99m}Tc
DP Revealed:

No definite evidence of active osseous deposits could be seen over the surveyed skeleton.
Degenerative changes are noted over the dorso-lumbar vertebrae.
Arthritic changes are noted over both shoulder and both knee joints.
Both kidneys are visualized.
Inclusion:
No definite evidence of active metastatic osseous deposits could be seen all over the surveyed skeleton.

Sh Obligated
Sia Talaat.MD

REPORT TEXT ENDE

جوستاف
روستاف
فمبادرة رئيس الجمهورية
لدعم صحة المرأة المصريةالهيئة المصرية
للتنظيم
الأسري

09/22/2026

MDT FORM

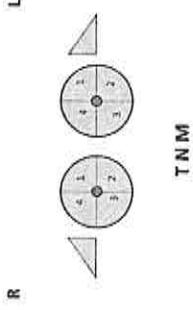
Name:	Heba Ali Ali Ahmed	MRN:	7939
Age:	50 Y	ID:	27605010103283
Menopausal Sta	prem	Phone:*	01200019831
NRS SCORE:		Marital Status:	
BMI:		Family History:	
Gender			
CO Morbidity:		Reason Of MDT:	
			newly developed mets in visceral crisis (dyspnea on mild exertion)
Surgical History:** rt mrm		ECOG Performance Status	
			☐ 0 ☒ 1 ☐ 2 ☐ 3 ☐ 4

Clinical Data:

BX:	
ER:	PR: HER2: KI67:

History:

premenop breast cancer with newly developed bone mets ,lun and mediastinal ln mets and newly developed large mediastinal mass
history : rt breast cancer upfront rt mrm 7/2022 t3 n3 13/16 mo idc er
8/8 pr8/8 her2-ve ki 30%
querry L5 lesion bs free pt st ht as she was presented to our hospital in 11/2022 st zoladex ai till now . recently dyspnea chest pain . cts 9/9/2026
: lung mediastinal mass (with suspected infiltration of SVC)involving precarinal lower rt paratracheal with extension to rt hilar region m 71x65,multiple bilat pulmonary nodules 15x11mm ,multiple med ln .
bs 17/9 multiple bone mets skull sternum ribs left shoulder left scapula DL VERT pelvic bone left femur rt tibia for mdt
cc dyspnea on mild exertion



Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Clinical Staging:

Sonomammography:

Biomarker/Molecular

Mets Workup:

Recommendations:

MDT Outcome & Plan

Start 2nd line Zoladex - Fulvestrant - Palbociclib
Bronchoscopic Biopsy from Rt hilar mass

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د. خالد عبد العزيز	د. محمد نوراني		

٢٠٢١/١١ ٢٧٦.٥.١.١.٣٢٨٣
حاصله على دبلوم تجاره
أنثى مسلمة
الشحات سعيد السيد ابراهيم
البطاقة سارية حتى ٢٠٢٨/١١/٢٧

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National ID Card

6100
Heba Ali Ali Ahmed
2026-09-19
Prof.Dr. no doctor
Goustave Roussey International Egypt

جوستاف
لوسساي
الانساني

جمهورية مصر العربية

بطاقة تحقيق الشخصية



هويه

على احمد بركات

٢٨ ش محمد الصاحب-هاشم اغا

الشرابية - القاهرة



٢٧٦.٥٠١.٠١.٣٢٨٣

JD3770610

Patient's Name: Heba Ali Ali Ahmed
Date: 2026-09-09
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST. ABDOMEN AND PELVIS REVEALED:

Known case of breast CA.

I- CHEST:

- Reduced volume of the right lower lung lobe showing multiple atelectatic bands.
- Large mediastinal mass lesion is seen involving the precarinal, lower right paratracheal region with extension to the right hilar region measuring 71 x 65 mm seen compressing the superior vena with suspected infiltration.
- Multiple bilateral scattered pulmonary nodules largest measuring 15 x 11 mm at the right upper lobe.
- Multiple mediastinal and bilateral hilar lymph nodes largest measuring 28 x 15 mm at the subcarinal region.
- No pleural or pericardial collection.
- Normal CT appearance of the heart.

II-ABDOMEN AND PELVIS:

- Enlarged liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal size, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- Multiple axial and appendicular skeleton-lytic bone lesions.

Opinion:

- ❖ Large mediastinal mass lesion with suspected infiltration of the superior vena cava.



SCAN & LABS

مركز النيل للأشعة و التحاليل

Name : Heba Ali Ali

Date : 12-09-2026 02:57

Branch : Dokki

Medical No.: 459951

Ref.Physician Prof : Goustav Roussey Egypt

تعاقد مستشفى : Ins.Company

Tc-99m MDP SKELETAL SCINTIGRAPHY

History:

50-year-old female patient with history of right breast cancer received radiotherapy and hormonal treatment, complaining of bone aches and referred for skeletal survey.

Technique:

Anterior and posterior whole-body scans 2 hours post injection of 20 mCi of the tracer.

Findings:

The surveyed skeleton is the seat of multiple, randomly distributed foci of variable tracer uptake keeping with bone secondaries. They are located at the skull bones (high parietal region), sternum, few bilateral scattered ribs, around left shoulder, inferior angle of left scapula, dorso-lumbar vertebrae, bony pelvis, trochanteric region of the left femur and proximal part of right tibia.

IMPRESSION:

- Multiple bone metastasis at the aforementioned sites.
- Follow up bone scan to monitor response to therapy.

Much obliged

Dr. Alaa Nagy, MD

Nuclear medicine

N I L E D I A G N O S T I C C E N T E R



Medical No. : 6100
Patient's Name: Heba Ali Ali Ahmed
Date: 2026-09-19
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

Right Digital soft tissue mammography with high resolution sonography revealed:

Clinical summary: Post right MRM & ALAND with muscle flap reconstruction, routine follow up.

Findings:

Left breast.

- Scattered fibroglandular densities. (ACR b)
- No speculations, masses, asymmetry, distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Right mastectomy bed.

- Clear mastectomy bed with no masses or fluid collections.
- Intact muscle with no masses or fluid collections.
- Clear left axilla. No pathological infra or supra-clavicular lymph nodes
-

Conclusion:-

- *Right breast (BIRADS 2): Clear right mastectomy bed and axilla with intact muscle flap.*
- *Left breast (BIRADS 1): normal study.*

Recommendation:-

- *For annual follow up.*

Best Regards,

DR. Amal Hegazi, M.Sc.

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

Category & recommendations	Suspected Cancer Risk
BIRADS 0: Non-informative study.	
BIRADS 1: Normal study, routine screening	0 %
BIRADS 2: Benign findings, routine screening	0 %
BIRADS 3: Probably benign, for close follow up.	2 %
BIRADS 4: Probably malignant, for biopsy.	50 %
A: mild suspicion.	
B: moderate suspicion.	
C: high suspicion.	
BIRADS 5: Malignant, for biopsy.	98-100%
BIRADS 6: Pathologically proven malignant	